



The health-productivity gap

Canada's untapped growth lever

Canada's universal health care system is one of our proudest national achievements, and our largest public investment.

It reflects a deeply held belief: that everyone deserves the opportunity to live a healthy life, and that caring for one another is central to who we are as a country. When the system frays or fails, the consequences are personal and far-reaching.

But the value of health extends well beyond its social benefits. When people are healthy, they can work, innovate, and build. When they are not, that potential is lost, and Canada's productivity suffers. Every traditional lever of productivity, including innovation, technology, and foreign capital, is muted when the workforce meant to power it is not healthy.

Deloitte's Health Productivity Gap (HPG) framework breaks down how improvements in our health care system can boost Canada's productivity, and outlines actions to achieve better outcomes.

By treating health as a productivity asset, we can target solutions that unlock that return. The opportunity is tens of billions of dollars in productivity gains—and improvements to Canadians' health and well-being that are priceless.



The challenge

Canada's health and productivity challenge

Productivity determines how much value workers create, and ultimately supports better wages, jobs, and living standards.

For more than a decade, Canada has lagged the *OECD* in average GDP per hour worked, a key measure of productivity. Canada's productivity in terms of GDP per hour worked¹, landed at US \$59.40 in 2024, which is roughly 29% below the United States' \$84.11.² Forecasts place Canada as the worst-performing advanced *OECD* economy over the next 40 years in real GDP per capita growth.³

While many of the causes of Canada's lackluster productivity, such as poor business investment, have been known for years, what hasn't been adequately discussed is the extent to which health care both contributes to the problem and offers solutions to mitigate it.

Like productivity, health outcomes have been falling. The share of adults reporting very good or excellent mental health fell from 72.4% in 2015 to 59% in 2021, with the steepest drop among young adults aged 18 to 34.⁴ The picture on overall health is no better: **after holding steady for two decades, the share of adults reporting very good or excellent health fell from 68.6% to 56.4% between 2015 and 2024**, with the sharpest decline again among young adults aged 18 to 34.⁵

Health-adjusted life expectancy (HALE), the number of years a person can expect to live in good health, has fallen by 3.5 years since its peak in 2012, erasing more than a decade of progress.⁶ Wait times have also worsened: the median wait from general practitioner referral to treatment has more than tripled, from 9.3 weeks in 1993 to 28.6 weeks in 2025.⁷ For a working-age Canadian, that wait isn't just stressful, it's expensive. The *Fraser Institute* estimated that waiting for care cost Canadian patients over \$4.2 billion in lost wages and productivity in 2025. That's an average of more than \$3,000 for each of the estimated 1.4 million Canadians in the queue.⁸

The impact of poor health on the productivity of the working-aged population extends beyond those who are sick to those with caregiving responsibilities.

Advanced economies like Canada require a vibrant workforce to boost productivity, the *OECD* stresses—one that is adaptable, mobile, skilled, and able to absorb change.⁹ However, people can't learn new skills and technologies, move, or change roles if illness has removed them from the workforce, or forced them to take time off to care for a loved one. Demographic change adds urgency: one in five Canadians is now over 65, driving up demand for chronic disease management, long-term care, and family caregiving.¹⁰ While many Canadians are healthier and working later in life, the sheer number of older adults is driving up chronic illness and the need for caregiving. The result is sustained pressure on both health spending and labour force participation, making productivity-enhancing health reform more urgent.

Canada's productivity agenda has focused on maximizing investment and technology while undervaluing the condition of human capital.¹¹ No return on investment, innovation, or reskilling can be maximized if the workforce isn't healthy enough to make use of it.

Canada is spending more on health than ever (likely about \$400 billion in 2025, or 12.7% of GDP¹²), yet ranks below average on many health metrics.¹³ While our values dictate treating health as a shared social commitment, it also can and should be managed as deliberately as any other driver of growth.

This is exactly what Deloitte's Health Productivity Gap framework is designed to help do.

Poor health affects productivity in ways that traditional metrics capture only partially, from reduced capacity at work to forgone participation, early exits from the labour force, and diminished long-term potential. The HPG estimates the economic output Canada loses when poor health limits people's ability to participate in the labour force. By translating these health-related impacts into dollars, the framework shows where targeted action could recover lost output and strengthen Canada's productive capacity—all while improving health.



The cost of illness

The Health Productivity Gap (HPG)

The HPG aims to answer two key questions:

How much productive capacity does Canada forgo when illness prevents full participation in the workforce? How much of that capacity could be recovered by those workers?

Using an established cost-of-illness (COI) framework, we capture the lost economic output through three lenses. We estimate that in 2025 alone Canada sacrificed more than \$100 billion in economic output across these areas:¹⁴

- **Caregiving costs:** providing unpaid care is associated with **\$48.5 billion** in lost labour income.
- **Morbidity costs:** living with a health condition is associated with **\$55.1 billion** in lost economic value from time away from work.
- **Mortality costs:** premature death due to a health condition is associated with **\$1.1 billion** in lost output during a “friction period” (the time required to replace and train workers).

Not all this lost economic output can be recovered.

Some sickness, waiting, and caregiving is unavoidable. But through targeted intervention aimed at reducing mortality, morbidity, and caregiving costs, much of this productivity and income loss can be regained, strengthening Canada’s overall productive capacity.

Economic gains through targeted interventions

Caregiving

The opportunity: Reducing the caregiving burden on families and informal carers by 25% could lead to economic gains of over \$12 billion.¹⁵

Why it’s achievable: A *Harvard Business School* study looked at employers offering a Care Concierge Benefit, which helped employees find services, coordinate care, and navigate the system. Absenteeism fell by up to 50%, and 30% of employees said caregiving support prevented them from taking time off or leaving their job. Employers realized up to 72% ROI from caregiver support programs.¹⁶ Notably, businesses have a direct stake in the health of their workforce, since healthier employees mean lower absenteeism, stronger retention, and sustained productivity.

Morbidity and mortality

The opportunity: Easing the morbidity burden that keeps people out of work or working below capacity by 25% could generate more than \$13 billion of GDP gains.¹⁷ Eliminating avoidable mortality (which *OECD* and *Statistics Canada* estimate as 25% of deaths in the working-age population) could add another \$250 million in GDP gains.¹⁸

Why it’s achievable: The *CMAJ Open* study estimates that about 40% of disability-adjusted life years (total health burden) and 24% of years lived with disability (morbidity burden) in Canada stem from modifiable risk factors, including diet, high BMI, blood pressure, and cholesterol.¹⁹ This suggests that improvements of over 25% are feasible by targeting preventable risk factors.²⁰

Closing the gap

Where Canada is losing productivity due to health

To realize Canada's untapped productivity opportunities, it helps to think of two underlying components:

The prevention gap:
Capacity lost to conditions that could have been prevented.

The participation gap:
Capacity drained by chronic disease, mental illness, and caregiving.

Closing these two structural gaps offers the greatest value: keeping people well, better supporting those already living with chronic conditions, and fostering higher workforce capacity.

The prevention gap: **Shift upstream to prevention and well-being to avoid loss of productive hours**

Upstream investments are most effective. These target the causes of illness, while downstream spending pays for hospitals, physicians, and treatments. The distinction matters. Preventing non-communicable disease delivers far greater social and economic returns than treating illness later, the *OECD* concludes. Avoidable chronic disease reduces participation, lowers productivity, and raises costs.²¹ The socio-economic returns for adult immunization alone, for example, are up to 19 times the initial investment.²²

Yet Canada's system remains biased toward downstream care. According to the *Canadian Institute for Health Information (CIHI)*, over half of total health spending in 2025 will flow to hospitals (26%), physicians (13.8%), and drugs (13.3%).²³ Most productivity, however, is gained or lost long before acute care is needed.

The call to shift care upstream is not new; it has been part of Canada's reform narrative for decades. What sets the opportunity apart now is recognizing that prevention is a productivity investment with a measurable return, and leaders' advantage lies less in how much they spend and more in how they design for it.

Singapore, for instance, builds prevention directly into personal health accounts rather than relying on public budgets alone. Every citizen holds a MediSave account, a mandatory personal health savings account that can be drawn on for care.²⁴

Historically oriented toward treatment, these accounts are now being deliberately repurposed for prevention. From 2027, the scheme is being renamed "MediSave Chronic and Preventive Care," withdrawal limits are rising, and national insurance is being extended for the first time to cover certain preventive procedures.²⁵ The design gives individuals a personal, portable stake in staying well, and makes prevention a standing feature of the system rather than a discretionary line that competes with acute care in every budget cycle.

The participation gap:

Tackle chronic disease, mental health, and caregiving to maximize wellness and keep people working at full capacity

Three forces combine to be the major productivity drag: chronic disease, mental health, and caregiving. Together, they reduce labour force participation, presence, and performance, and impair the quality of life for millions of Canadians. Currently, these pressures are trending in the wrong direction:

Chronic disease

The incidence of chronic disease keeps rising. In 2023, nearly half of Canadian adults lived with one or more chronic disease (46.1%), up from 41.2% in 2015.²⁶ Chronic disease treatment accounts for roughly 58% of annual health care costs.²⁷

Mental health

Mental health is the largest driver of long-term disability (40% of claims in 2024), with depression claims up 33% and anxiety claims up 50% in 2025 when compared with pre-2020 levels.²⁸ Substance use compounds these impacts. In construction, for example, up to 33% of personnel in safety-sensitive roles report being impaired, hungover, or using substances shortly before or during work.²⁹ (The broader impact of poor mental health costs the Canadian economy indirectly as well, from housing supports, to demand for police and correctional services.)

Caregiving

Caregiving adds a third burden. Nearly eight million Canadians provide unpaid care, and most working caregivers report reduced hours, earnings, or advancement.³⁰ This represents the largest lever available for lifting workforce participation and output.



The solution

Seven integrated actions

What Canada needs to get right in a shifting world

To narrow these gaps, Canada should pursue and track a set of integrated actions that directly improve the country's HPG. There's no time to waste; an aging population, rising chronic disease, and a rapidly shifting technological and geopolitical landscape necessitate urgent action.

The following seven actions aim to rebuild the system around a single organizing idea: in addition to being an individual and social good, health is a productivity asset. The first four restore people's capacity to participate; the next three expand the system's capacity to deliver. Each reinforces the others.

Action 1

Enroll every Canadian in a Health Home

Canada's system is built around episodes of poor health during which reliable, consistent relationships with medical professionals are sadly lacking. Approximately six million people have no family doctor.³¹ A Health Home integrates primary care and prevention, giving every person a named care team as their front door to care. The Netherlands shows what universal attachment looks like: more than 95% of residents have a family doctor, sustained by a system that obligates insurers to attach everyone to a practice and keep them enrolled over time.³² That continuity is not just convenient; in Denmark, adults affiliated with the same practice for a decade or more have 21% lower mortality than those recently listed.³³

But enrollment alone is not enough; the Canadian model must translate continuity into measurable prevention outcomes. Alongside a named team, a digital health plan, and a funded prevention package, a share of each Health Home's funding should be tied to defined prevention outcomes. These include: screening and immunization rates, risk-factor control for conditions like hypertension and diabetes, and enrolled-population health gains. All need to be reported transparently

and benchmarked from practice to practice. Incentives work; in England's pay-for-performance model, for example, recorded prevention outcomes improved when payments were linked to performance and fell by 11 to 13 percentage points when withdrawn.³⁴ Canada should combine incentives with accountability, rewarding teams that keep people healthy and holding back funding where targets go unmet, so prevention becomes a financed, measured promise rather than an aspiration.

Action 2

Make prevention proactive and personalized

Once prevention is prioritized, it can become more proactive, personalized, and continuous. Canada should move beyond periodic checkups and broad public health campaigns to identify risks earlier and intervene sooner. England's *NHS Diabetes Prevention Programme* identified people with prediabetes and enrolled them in a behaviour-change course. Participants were 20% less likely to develop type 2 diabetes within three years.³⁵ Canada should move beyond isolated pilot programs and systematically deploy targeted prevention initiatives for major drivers of productivity loss, including diabetes, cardiovascular disease, obesity, mental health, and musculoskeletal conditions.

Technology can help make prevention continuous rather than episodic. In one randomized trial of over 100,000 women, AI-assisted mammography detected 29% more cancers and almost halved radiologists' workload (44% reduction), without increasing interval cancers.³⁶ Emerging tools go even further: in eye exams, AI reading of a routine retinal photo flags cardiovascular risk with about 91% sensitivity.³⁷ Combined with longitudinal health data, these tools allow care teams to identify people at risk before symptoms emerge and intervene when prevention is most effective.

Action 3

Remove barriers to participation

Good physical health only fosters productivity if people are in a position to act on it. Two groups are systematically held back: caregivers and those with mental health challenges.

Family caregivers would benefit from better support through federal measures, such as broadening the existing EI caregiving benefit beyond its current critical-illness and end-of-life eligibility to include chronic care, paired with respite, home-care hours, and job-protected leave.³⁸ Second, legislate a mental health and substance use parity so these conditions are funded equitably with physical health, and add rapid-access therapy, prevention, and early intervention to every workplace.

Action 4

Give individuals financial tools for prevention

System financing should reach individuals as well as institutions. By establishing a universal Prevention Savings Account (PSA), individuals can access a portable, incentive-linked entitlement to invest in screening, risk-factor management, and healthy behaviours before illness begins. The idea draws on Singapore's account-based model, but the mechanism is Canadian, building on Health Spending Accounts already offered by many employers and extended toward universality through a public seed or match.

Designed as a complement to Medicare, it sits outside the Canada Health Act's insured services and is most workable as a federal tax-and-transfer instrument. Unlike earlier medical savings account proposals framed around cost-containment, its purpose is prevention and productivity, turning prevention from a cost that governments trim into an asset that individuals own.

Action 5

Create capacity from what we already have

Canada should unlock wasted capacity, reinvesting it into Health Homes staffed by interdisciplinary teams with upstream care targets.

Physicians lose about 19.8 million hours a year to unnecessary administration—the equivalent of approximately 9,000 full-time doctors.³⁹ Federal, provincial, municipal, and regulatory bodies

should adopt a shared target to cut this burden (for example, by 10% within three years). Each level can act on the levers it controls, from billing and reporting requirements to medical college documentation rules, and deploy AI to reclaim the hours.

Kaiser Permanente's AI Scribe technology alone saved about 15,800 physician hours in a year, and agentic AI platforms can take over entire back-office functions.⁴⁰

Hospitals, too, can become capacity engines. Canadian hospitals run at roughly 91% occupancy rate,⁴¹ yet 5-10% of inpatients could be safely cared for at home.⁴² Scaling Ontario's *Hospital-at-Home* program nationally would ease that pressure. For example, Bayshore's @home programs, with 13 Ontario hospitals, have freed more than 240,000 alternate-level-of-care bed days, with 94% of patients recommending the program.⁴³

Action 6

Meaningfully digitize health infrastructure

None of this can be scaled up without data that moves with patients. Bill S-5 (the Connected Care for Canadians Act) is proposed to mandate common interoperability standards and prohibits data blocking by health information technology vendors.⁴⁴ We can move fast and make progress visible through a public scorecard (share of providers exchanging structured data), with the goal of 100% interoperability.

Action 7

Redesign financing and incentives

Make outcomes, not just inputs, the shared measure of success for federal and provincial health funding. Building on existing agreements, federal and provincial governments could link a portion of health transfers to jointly agreed progress measures. Provinces would receive predictable base funding alongside shared-outcome funding tied to improvement against a pan-Canadian productivity scorecard. That scorecard could be co-developed with provinces, anchored in the opportunities identified here, and housed in an arm's-length body such as *CIHI* to keep metrics credible and independent.

What's next

How to move forward

Canada's productivity problem is also a health problem. For decades, we've managed the two as separate files and Canada has paid twice: in worse health outcomes but also in lost economic output.

Through collaborative efforts, targeted improvements to health care could add over **\$25 billion to the Canadian economy**—productivity reforms that benefit citizens in multiple ways.

By tracking the prevention and participation gaps, we can target investments where the productivity return is greatest, leveraging a certain portion of health spending to do more. The HPG gives governments, health care organizations, employers, and other stakeholders insights to act on, and a clear imperative: consider workforce health and productivity alongside every other driver of growth.

Get this right, and we don't just fix health care. We redefine what the economy is capable of, shaping how Canadians live and work, organizations perform, and Canada competes. We unleash the untapped productive capacity of our people, while improving a health care system that is core to the values and well-being of all Canadians.



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Contact us



Michelle Theroux
National Health Leader,
Deloitte Canada
mtheroux@deloitte.ca



Dr. Tom Stewart
Chief Medical Officer,
Deloitte Canada
thstewart@deloitte.ca



Matthew Stewart
Partner, Economic Advisory,
Deloitte Canada
mastewart@deloitte.ca



Geneviève Giguère
Quebec Health Leader,
Deloitte Canada
gegiguere@deloitte.ca

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